

Cory Lea Shepherd
Administration Officer (AO3), Switchboard Services, Logan Hospital
0417 400 227 | coryshepherd1@hotmail.com

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Michelle Harrison
Injury Management Consultant, LBHS HR Team 1
Human Resources, Logan and Beaudesert Hospital and Health Service
Metro South Health

cc: Lyndelle Forrest, Senior Human Resources Consultant

RESPONSE TO CORRESPONDENCE OF 29 JULY 2026 - REQUEST FOR FURTHER MEDICAL INFORMATION
Employee Capability Checklist dated 3 July 2026 (MSH-INJ-5795)

Dear Michelle,

Thank you for your email of 29 July 2026.

1. MY ELECTION

1.1 I will take the request to my treating practitioner myself. I would rather progress this quickly than add a further step, and I will arrange an appointment as soon as I have the request.

1.2 As I will provide the request to Dr Ma directly, I do not propose to complete the Employee Authority Form. It is broader than the present request - it extends to disclosures to and from WorkCover Queensland and income protection insurers, and it is open-ended in duration. If a written authority is needed for Dr Ma to respond to a specific request, I am happy to provide one confined to that purpose.

2. WHAT I NEED IN ORDER TO DO THAT

2.1 To put a request to Dr Ma, I need the request. Your email states that "further clarification is required regarding your functional capacity and the medical restrictions identified", but does not identify what is unclear. Dr Ma cannot respond to a request in those terms.

2.2 Could you please provide, in writing:

- (a) the specific questions the Health Service asks Dr Ma to answer, set out as questions;
- (b) for each question, the duty or task to which it relates, identified by reference to the Administration Officer, Switchboard Services (AO3) role description, which Dr Ma reviewed before completing the checklist;
- (c) what in the 3 July Employee Capability Checklist is said to be insufficient - which entry requires clarification, and why the existing entry does not enable the assessment; and
- (d) who will consider Dr Ma's response, and the timeframe within which the decision about my duties will then be made.

3. WHY THAT SPECIFICITY IS NEEDED

3.1 Your email states the request is made under the Health Service's obligation to assess the medical information "alongside the inherent requirements of the role". I accept that the Health Service has its own obligations. Five things follow.

(a) The assessment is task-based, so the questions should be. QH-POL-210 directs adjustments to enabling an employee to safely perform the genuine occupational requirements of the role, tailored to their circumstances, and reflects the obligation under the Anti-Discrimination Act 1991 (Qld) to make reasonable adjustments for an employee with an impairment unless doing so would impose unjustifiable hardship. QH-IMP-401-5 makes managers responsible for identifying suitable duties, guided by medical information. Each operates at the level of identified duties. A refusal of all duties, without identifying which duty cannot be performed or adjusted, does not engage that assessment - and a question not tied to a duty cannot produce information that either instrument can use.

(b) Assessing a risk requires identifying the risk. EB12 clause 7.2 and the Managing the risk of psychosocial hazards at work Code of Practice 2022 proceed by identifying a hazard, assessing the risk it presents, and considering controls. I am not presently able to identify what risk is said to arise, or from which duty - and without that I cannot meaningfully participate in the assessment, as consultation under sections 47 to 49 of the Work Health and Safety Act 2011 (Qld) contemplates.

(c) The checklist is the Health Service's own instrument. It was required by the Health Service and completed by Dr Ma after he reviewed the AO3 role description. If it is now insufficient, the entry said to be insufficient should be identifiable, together with what the form did not capture.

(d) The arrangement is not new. I worked in the role on reduced hours within my certified capacity for approximately twelve months before 3 July 2026, and the Health Service's correspondence of 7 July 2026 records that my restrictions were already being accommodated. Could the request please also identify what has changed - that is, which duty is now said to be affected in a way it was not during that period.

(e) The framework identifies hazards in the work, not in the worker. The Code of Practice identifies psychosocial hazards by reference to features of work and its management - job demands, job control, support, role clarity, change management, workplace relationships and organisational justice. EB12 clause 7.2 correspondingly provides for risk assessments of workplaces, conducted using the Code and in consultation with workers and health and safety representatives. I therefore ask that any assessment framed in psychosocial terms be directed to the work and the work system, and that questions to Dr Ma be confined to my capacity to perform identified duties.

4. THE RESTRICTIONS AND THE DUTIES

4.1 Since your email refers to the inherent requirements of the role, I have compared the restrictions certified on 3 July with the responsibilities set out in the AO3 role description.

4.2 The role description lists the position's key responsibilities as: attending to telephone enquiries and maintaining call queues; collating information and maintaining the Omnivista database and SharePoint; supporting the allocation, coordination and fault repair of pager units; participating in the emergency response process in accordance with emergency code procedures; establishing and maintaining networks and records; exercising discretion and judgement to resolve problems within the scope of the role; interacting appropriately with the public and service providers; multitasking and operating under pressure; contributing to the team; working with limited supervision; treating patients and staff fairly; and following safety policies and procedures.

4.3 First, the restriction on complaint-handling does not correspond to any responsibility listed in the role description. If the Health Service considers complaint-handling to be an inherent requirement of the position, could you please identify where it appears in the

role description, or on what other basis it is said to be an inherent requirement.

4.4 Second, the remaining restrictions - the limits on peak weekday day and afternoon shifts, predictable rostering, and a minimum ten-hour break between shifts - go to when I work, not to which duties I perform. They do not remove any of the responsibilities listed above.

4.5 I note the role description records that the position is a continuous shift working role. I worked that role, on reduced hours and within my certified restrictions, for approximately twelve months prior to 3 July 2026, and the correspondence of 7 July 2026 records that those restrictions were already being accommodated. If the rostering restrictions are now said to be incompatible with the role, could you please identify what has changed since that period.

4.6 Third, a reduction in ordinary hours is an arrangement the industrial instruments expressly contemplate. Clause 8.2 of the Hospital and Health Service General Employees (Queensland Health) Award - State 2015 provides for part-time employment on a regular number of ordinary hours, pro rata to full-time conditions, with a minimum of eight ordinary hours per fortnight. EB12 clause 10.3 provides that an employee may request a change in the way they work, including their ordinary hours, that such a request may be refused only on reasonable grounds, and that the employer must give written notice of its decision, with reasons, within 21 days.

4.7 A reduction in hours does not of itself remove any duty from the role, and where such an arrangement is made for reasons unrelated to health it does not ordinarily require medical clarification or absence from the workplace while it is considered. Could the Health Service please identify why the reduced hours certified on 3 July 2026 are treated differently - that is, what it is about those hours, as distinct from the reasons for them, that prevents the duties of the position from being performed.

4.8 Accordingly, could the request to Dr Ma please identify which of the responsibilities listed at paragraph 4.2 the Health Service says I cannot perform, and which restriction is said to prevent it.

5. THE CHARACTERISATION OF RISK

5.1 Across your correspondence of 13, 15 and 29 July 2026, the Health Service has stated that it must assess whether my return can be "safely and operationally supported", whether "the matters identified by Dr Ma ... present any psychosocial hazards or risks", whether duties "can be safely facilitated", and has referred to its obligation to ensure the health and safety of workers "and others in the workplace".

5.2 I appreciate that the last of these reflects the language of the primary duty, and I do not suggest the Health Service should not satisfy itself as to safety.

5.3 However, in the twenty-seven days since 3 July, no incident, complaint, hazard or particular of any kind has been identified to me. Nor was any raised at any point during the approximately twelve months I worked in the role on reduced hours, or in the correspondence of 7 July 2026 recording that my restrictions were already being accommodated. There has been no finding of any kind made in relation to my performance of this role.

5.4 A concern about safety expressed only in general terms is not something I am able to answer, and it is not something Dr Ma can answer either. Could you please therefore confirm:

(a) whether it is asserted that my performing the duties of my position, within the restrictions certified on 3 July 2026, would present a risk to my own health and safety or

to that of any other person; and

(b) if so, the basis for that view - the duty concerned, the risk identified, and the material relied upon.

5.5 If no such assertion is intended, I would be grateful if you would confirm that in writing, so that the questions to Dr Ma can be confined to my capacity and restrictions, and so that the position is clear on my record.

6. SCOPE

6.1 I will provide Dr Ma with the questions and the role description. My consent extends to information concerning my capacity, my restrictions, and their operational application to the duties of my position, and I ask that the questions be limited to what is necessary for that purpose. That is consistent with the Health Service's own collection notice, which records that information of this kind is collected to support rehabilitation and return to work.

7. TIMEFRAME

7.1 Given the request has been in contemplation since 3 July, could the questions please be provided within two business days. On receipt I will provide them to Dr Ma promptly, confirm to you the date I have done so, and forward his response as soon as I receive it.

7.2 If it would assist, I am willing to meet at short notice to work through the role description and the certified restrictions duty by duty. I would ask that any meeting occur alongside, rather than in place of, a written response.

8. OTHER MATTERS

8.1 I note your email does not respond to the matters raised in my emails of 28 July 2026, which remain outstanding within the period identified in that correspondence. I do not repeat them here.

8.2 I remain certified fit, ready and available to work within the 3 July 2026 Employee Capability Checklist.

Yours sincerely

Cory Lea Shepherd
Administration Officer (AO3), Switchboard Services
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